Universal Health Insurance

A Hybrid Framework for Universal Coverage

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Executive Summary

Healthcare remains one of the most significant financial, economic, and social challenges facing the United States.

Millions of Americans rely on employer-sponsored insurance, government programs, private plans, or some combination of coverage systems. Despite substantial national healthcare spending, many individuals continue to face high out-of-pocket costs, delayed treatment, limited access to care, or financial hardship resulting from illness and injury.

This paper proposes a Universal Health Insurance (UHI) framework intended to provide comprehensive healthcare coverage while preserving patient choice, encouraging innovation, and maintaining a role for supplemental private insurance.

The central principle of this paper is straightforward:

HEALTHCARE SHOULD BE UNIVERSAL; NO PERSON SHOULD FACE FINANCIAL RUIN BECAUSE OF MEDICAL CARE.

The proposal seeks to achieve those goals through:

- Universal comprehensive healthcare coverage.
- A publicly financed baseline insurance system.
- No copays or deductibles for covered care.
- Optional supplemental private insurance.
- National drug-price negotiation.
- Strong support for rural healthcare access.
- Phased implementation and continuous evaluation.

- Independent fiscal and economic analysis.

This paper does not claim to provide a final financing model or complete cost estimate. Rather, it presents a framework intended for review, criticism, and refinement by healthcare economists, physicians, administrators, policymakers, and the public.

Introduction

Healthcare is unlike most consumer goods.

Individuals rarely choose when serious illness, injury, cancer, chronic disease, or medical emergencies occur. Access to healthcare can affect survival, quality of life, economic productivity, educational outcomes, family stability, and long-term public expenditures.

The United States spent $5.3 trillion on healthcare in 2024, or $15,474 per person and 18.0 percent of GDP [1], yet many citizens continue to face significant barriers to care.

This proposal begins with simple premises:

Healthcare should not depend on employment status, income, geographic location, or preexisting medical conditions.

The purpose of Universal Health Insurance is not to eliminate all private healthcare options.

The purpose is to guarantee healthcare access for all while preserving flexibility, competition, and innovation where they provide value.

The Challenge

Despite that spending, many Americans continue to face barriers to care, financial hardship, and uneven access to essential health services.

The American healthcare system delivers world-class treatment in many areas and drives significant medical innovation. However, several persistent challenges remain:

- High out-of-pocket costs.

- Medical debt.

- Gaps in insurance coverage.

- Complex administrative systems.

- Rural provider shortages.

- Mental-health access limitations.

- Rising prescription-drug costs.

- Coverage tied to employment.

These challenges create costs that extend beyond healthcare itself.

When individuals delay care because of cost:

- Conditions often worsen.

- Treatment becomes more expensive.

- Emergency care utilization increases.

- Productivity declines.

- Public expenditures frequently rise later.

Similarly, untreated mental-health conditions, lack of prescription access, neglected dental care, and delayed preventive services can eventually develop into larger and more expensive problems.

Healthcare policy should therefore consider both the cost of providing care and the cost of failing to provide it.

Several characteristics define the current healthcare landscape:

- Millions of Americans remain uninsured or underinsured.

- Medical debt remains a significant source of financial distress.

- Rural hospitals and healthcare providers face growing financial pressure.

- Mental-health and behavioral-health services remain difficult to access in many regions.

- Employer-sponsored insurance remains the primary source of coverage for many Americans, leaving coverage vulnerable to job changes and economic disruptions.

- Administrative complexity increases costs for patients, providers, insurers, and government programs.

These realities do not necessarily prove that any particular reform is correct. They do demonstrate that the healthcare system faces challenges that warrant continued evaluation and reform.

The central question is therefore not simply whether Americans spend enough on healthcare. The central question is whether healthcare resources are being allocated in a manner that maximizes access, quality, affordability, and long-term health outcomes.

This paper proposes Universal Health Insurance as one possible framework for addressing those challenges.

The Baseline: What the United States Spends

Federal data provide a starting point for evaluating this proposal [1][2]:

National health spending reached $5.3 trillion in 2024, or $15,474 per person, and accounted for 18.0 percent of GDP [1].

Private health insurance accounted for $1.64 trillion (31 percent), Medicare $1.12 trillion (21 percent), Medicaid $931.7 billion (18 percent), and out-of-pocket spending $556.6 billion (11 percent) [1].

CMS projects that health spending will grow 5.4 percent a year over 2025 to 2034, against 4.1 percent for GDP, raising health's share of the economy from 18.0 percent in 2024 to 20.6 percent in 2034 [1].

CMS projects the insured share of the population to fall from 91.8 percent in 2024 to 90.5 percent in 2034 [1].

Medicare's Hospital Insurance trust fund is projected to deplete its reserves in the second quarter of 2033 [2].

Private insurance premiums, employer contributions, and out-of-pocket payments together form the pool that a universal system would partly redirect. Whether total spending would rise or fall is a question for independent analysis.

Guiding Principles

Principle 1: Universal Coverage

Every individual should have access to healthcare coverage regardless of employment status, income, age, disability, or geographic location.

Principle 2: Financial Protection

No person should face bankruptcy, severe financial hardship, or long-term economic instability because of necessary medical care.

Principle 3: Consumer Choice

Citizens should retain the ability to purchase supplemental private insurance.

Principle 4: Innovation

Healthcare reform should preserve incentives for medical research, technological advancement, pharmaceutical development, and improved treatments.

Principle 5: Rural Access

Healthcare availability should not depend on ZIP code.

Principle 6: Fiscal Responsibility

Healthcare reforms should undergo independent cost analysis, phased implementation, and ongoing evaluation.

Universal Health Insurance Structure

This proposal establishes a publicly financed Universal Health Insurance system available to all citizens, lawful permanent residents, and other lawfully present residents. Emergency care, treatment of communicable disease, and public-health services would be available to all persons regardless of immigration status.

Unlike some single-payer proposals, this framework does not prohibit private insurance.

Instead, it seeks to combine universal coverage with consumer choice.

The proposed structure consists of two layers:

Universal Health Insurance

A publicly financed insurance system that provides comprehensive healthcare coverage to every eligible resident.

Supplemental Insurance

Individuals may purchase additional insurance for services beyond the standard UHI package, upgraded accommodations, or other supplemental benefits. Private insurance would supplement, and not replace, the public plan.

The objective is to ensure that nobody lacks healthcare while still allowing market competition and personal choice.

Comprehensive Coverage

Universal Health Insurance would provide comprehensive coverage for medically necessary care.

Covered services would include:

- Primary care
- Specialist care
- Preventive care
- Emergency care
- Hospitalization
- Surgical care
- Mental health treatment
- Substance-use treatment
- Prescription drugs
- Dental care
- Vision care
- Hearing care
- Home health services
- Long-term care
- Fertility treatment

- Rehabilitation services
- Reproductive and maternal health, including contraception, pregnancy and postpartum care, and abortion care (coverage terms in Platform Section 6.2; conscience protections, the Patient Navigation Service, and emergency-care requirements in Platform Section 6.3; financed only through the Universal Health Insurance Trust Fund)

The rationale is straightforward.

Many services currently considered separate categories eventually become broader healthcare problems when left untreated.

For example:

- Untreated dental issues can become serious medical conditions.
- Untreated mental illness can lead to emergency intervention.
- Lack of prescription access may result in hospitalization.
- Delayed preventative care often leads to higher long-term costs.

Comprehensive coverage seeks to address healthcare needs earlier rather than waiting until they become more expensive crises.

Why Comprehensive Coverage?

This proposal rejects a narrow insurance model that limits coverage to catastrophic medical events.

Many healthcare services traditionally excluded from comprehensive coverage, including dental care, mental healthcare, hearing care, prescription access, and preventative services, can create significantly larger health and fiscal costs when left untreated.

The objective of comprehensive coverage is not merely to pay for healthcare after a crisis occurs. The objective is to intervene earlier when treatment is often more effective and less expensive.

Whether comprehensive coverage produces net savings remains an open question requiring independent analysis.

No Copays or Deductibles

Covered services would carry no copays or deductibles at the point of care. This removes cost as a barrier to care and supports the principle that no person should face financial ruin because of medical care.

Eliminating cost sharing also removes a common check on utilization. Provider payment design, workforce capacity, and spending-growth controls therefore matter more, and the effect on total spending is a question for independent analysis.

Financing Framework

Universal Health Insurance would be financed through a combination of:

- Payroll contributions.
- Income-based taxes.
- Employer contributions.

Wealth-tax revenue would not be used for health financing. Under the platform's financing rules, each revenue stream has exactly one use. Wealth-tax revenue first fills the Sovereign Wealth Reserve for recessions, and only a smoothed share reaches later-tier General Fund programs; none of it reaches the UHI Trust Fund.

The proposal does not yet specify exact contribution rates. The platform deliberately leaves them for independent scoring and expert review rather than setting rates before the costs are known.

The platform's own costing (Appendix E.4 of the full platform) shows the scale of the question. Comparable single-payer designs that include long-term care would add about $32.6 trillion (Mercatus Center) to $34 trillion (Urban Institute) to federal spending over ten years. Much of that would replace premiums, deductibles, and state Medicaid spending rather than add to national health spending: the Congressional Budget Office's 2020 options found national health spending in 2030 ranging from about $0.7 trillion lower to $0.3 trillion higher than under current law. Even so, the Trust Fund would need to raise roughly $3 trillion or more a year.

Any financing design must work within four fixed constraints:

- **The wage ceiling.** Combined federal tax rates on wages may not exceed 60 percent (Section 2.8 of the platform), which leaves about 4.2 percent for the UHI contribution on wages above the top income-tax bracket threshold.
- **No General Fund support.** A shortfall slows UHI's phase-in; it is never covered from the General Fund or another revenue stream.
- **Long-term care and eldercare are included.** Care for older adults is part of the long-term care benefit, with room-and-board costs capped by income (Section 5.1.1).
- **Savings lower rates.** Savings from consolidated health financing and the share of drug-negotiation savings not assigned to the Innovation Fund lower contribution rates and are counted nowhere else.

However, the framework begins from an important observation.

Americans already spend substantial sums on:

- Insurance premiums.
- Employer healthcare contributions.
- Deductibles.
- Copays.

- Coinsurance.
- Out-of-network expenses.
- Medical debt payments.

To many households and businesses, healthcare costs already function as unavoidable expenditures.

This proposal explores whether a portion of those existing expenditures can be redirected into a universal financing system that guarantees coverage for everyone. In practical terms, many existing healthcare expenditures could potentially shift from private premiums, deductibles, and out-of-pocket costs toward a more predictable tax-supported financing model. Whether such a transition would increase or decrease total national healthcare spending remains a question requiring detailed economic analysis.

The key question is not simply:

How much would UHI cost?

But also:

How much healthcare spending is already occurring, and how efficiently is it currently being used?

Provider Payments

Healthcare providers would remain independent.

Hospitals, clinics, physicians, and healthcare systems would continue delivering care.

This proposal does not advocate nationalizing healthcare providers.

Instead, provider payments would follow a national fee schedule set by an independent commission, using Medicare's relative-value weighting as a foundation. A data-driven appeals process would allow specialties and regions to challenge rates so that errors can be corrected before shortages or closures occur.

The objective is stability during transition.

Medicare-based rates are lower than what private insurers pay hospitals today. In 2022, employers and private insurers paid hospitals an average of 254 percent of what Medicare would have paid for the same services [3]. Moving toward Medicare-based rates therefore needs to be gradual and monitored for access, closures, and workforce effects.

Potential reforms beyond that point should be evaluated based on:

- Provider participation.
- Patient outcomes.
- Rural access.

- Administrative burden.
- Fiscal sustainability.

Pharmaceutical Policy

Prescription-drug costs represent a major component of healthcare spending.

This proposal advocates:

National Price Negotiation

The UHI system should negotiate pharmaceutical prices to maximize value for taxpayers and patients. Price negotiation should focus on obtaining value for taxpayers and patients while maintaining incentives for pharmaceutical research, development, and innovation.

Medicare already negotiates prices for selected drugs under current law: negotiated prices for the first ten Part D drugs took effect on January 1, 2026 [5]. This proposal would extend negotiation to the UHI system.

Continued Innovation

Drug-development incentives should remain strong.

Innovation remains essential for improving healthcare outcomes.

Variable Exclusivity and a Public Patent Option

New therapies would receive market exclusivity of 7 to 15 years, based on research investment, therapeutic benefit, and public-health impact, followed by a public patent option that allows public manufacturing rights and generic competition.

Generic Competition

Generic competition should be encouraged whenever exclusivity periods expire.

Private Manufacturing

Pharmaceutical production should remain primarily within the private sector.

Strategic Public Procurement

Government may contract private manufacturers to produce medicines necessary to meet public-health needs when shortages or access problems occur.

The objective is balancing innovation with affordability.

Rural Healthcare Stability

Coverage alone is insufficient if healthcare providers are unavailable.

Rural healthcare systems face unique challenges:

- Smaller patient populations.
- Staffing shortages.
- Geographic isolation.
- Financial instability.

This proposal supports:

- Rural Stabilization Grants
- Targeted funding to maintain essential services.
- Critical Access Protections
- Measures designed to reduce closures of rural hospitals and clinics.
- Minimum Service Guarantees
- Basic healthcare services should remain available regardless of geographic location.
- Geographic Reimbursement Adjustments
- Payment systems should account for rural operating realities.
- Workforce Capacity

Loan forgiveness and training incentives would support physicians, dentists, optometrists, nurses, and behavioral-health providers who serve underserved and rural communities, paired with expanded training capacity.

Healthcare access should not depend on where someone lives.

Why Preserve Private Choice?

Many universal healthcare systems rely almost entirely on a single public insurer.

This proposal takes a hybrid approach.

The goal is not government monopoly.

The goal is universal coverage.

Private providers would continue to deliver care, and private insurers could offer supplemental coverage alongside UHI. Keeping the public plan as the single source of baseline coverage avoids the risk-selection and overpayment problems seen when private plans substitute for public coverage. For example, the Medicare Payment Advisory Commission estimates that Medicare will spend 14 percent, or about $76 billion, more in 2026 on Medicare Advantage enrollers than it would on the same beneficiaries in traditional Medicare, largely because of favorable selection and coding intensity [4].

Allowing private providers and supplemental coverage may:

- Increase individual flexibility.
- Encourage innovation.
- Provide additional consumer options.
- Reduce political concerns regarding centralized control.

At the same time, universal public coverage ensures that healthcare access is not dependent on employment or personal wealth.

Why Not Employer-Based Insurance?

Employer-sponsored coverage has provided insurance to millions of Americans and remains an important part of the current healthcare system.

However, tying healthcare access to employment creates vulnerabilities.

Coverage may change due to:

- Job loss.

- Career transitions.

- Economic downturns.

- Employer decisions.

This proposal seeks to separate healthcare access from employment status while still preserving a role for employers in financing the healthcare system.

The objective is not to eliminate employer contributions. The objective is to ensure that healthcare coverage remains stable regardless of labor-market changes.

Implementation Strategy

Healthcare reform of this scale should not occur overnight.

Major transitions create uncertainty and implementation risks.

This proposal therefore favors phased implementation.

Potential steps include:

- **Phase 1**
  - Expanded Medicare buy-in options.
- **Phase 2**
  - Coverage expansion for children.
- **Phase 3**

    - Gradual expansion into broader age groups.
- **Phase 4**
    - Universal enrollment.

At every stage:

- Costs should be evaluated.
- Outcomes measured.
- Problems identified.
- Adjustments made.

The objective is successful implementation rather than rapid implementation.

Because Medicare's Hospital Insurance trust fund is projected to run out of reserves in the second quarter of 2033 [2], a Medicare-based first phase would need its own financing.

Implementing legislation would coordinate Medicare, Medicaid, veterans' health programs, and employer-sponsored coverage so that no one loses coverage during the transition.

Areas Requiring Further Research

Significant questions remain unresolved.

These include:

- Total program cost.
- Revenue requirements.
- Provider reimbursement effects.
- Labor-market impacts.
- Administrative savings.
- Transition costs.
- Workforce capacity.
- Rural healthcare sustainability.
- Long-term fiscal projections.
- Interaction with supplemental private insurance markets.

These areas require independent study before implementation.

Open Policy Questions

Several important questions remain open.

Financing

What combination of payroll contributions, employer contributions, income-based taxes, state compact payments, and healthcare savings would be required? Specific questions for reviewers:

- What contribution rates would balance the Trust Fund over ten years and over the long term, within the 60 percent wage ceiling?
- How should current employer premium spending convert into employer contributions, and how should wages adjust as premiums end?
- How should the burden be shared between payroll and income-based contributions so that middle-income households pay less than they now pay in premiums and out-of-pocket costs?
- How fast can the system phase in if revenue falls short, given that it may not draw on the General Fund?

Provider Payment

How should the independent commission set and adjust rates, and how quickly should rates move toward Medicare-based levels?

Supplemental Insurance

What rules should govern supplemental coverage, including whether it may duplicate services covered by UHI?

Utilization Without Cost Sharing

What mechanisms should manage utilization and spending growth when covered care carries no copays or deductibles?

Long-Term Care

What benefit structure, including eldercare and the income-based caps on room and board, would be sustainable over multiple decades?

Workforce Supply

How can physician, nursing, and behavioral-health shortages be addressed?

This paper intentionally leaves these questions open for further discussion.

Policy Tradeoffs and Risks

Healthcare reform inevitably involves tradeoffs.

This proposal does not assume that universal coverage automatically produces better outcomes in every circumstance. Instead, it seeks to balance competing priorities while making those tradeoffs explicit.

Comprehensive Coverage

Comprehensive coverage may improve access to preventative care, mental-health treatment, prescription medications, dental services, and other essential care.

However, broader coverage also increases program scope and potentially increases public expenditure.

The extent to which earlier treatment offsets those additional costs remains an important question requiring independent analysis.

Eliminating copays and deductibles removes cost as a barrier to care but also removes a common check on utilization, which makes provider payment design and workforce capacity more important.

Pharmaceutical Pricing

National price negotiations may reduce prescription-drug expenditures for patients and taxpayers.

However, lower revenues may affect research and development incentives within the pharmaceutical industry.

An effective system should seek to lower costs while preserving strong incentives for medical innovation.

Universal Access

Universal healthcare coverage may reduce financial barriers to treatment and improve continuity of care.

At the same time, expanding access could increase demand for healthcare services.

Potential consequences may include:

- Workforce shortages.

- Provider-capacity constraints.

- Longer wait times in some regions.

- Increased pressure on rural healthcare systems.

Addressing those risks will require workforce development, provider participation, and careful implementation planning.

Recognizing these tradeoffs is essential to evaluating whether Universal Health Insurance would achieve its intended goals.

Transition Risk

Any large-scale healthcare reform faces implementation risks.

Employers, insurers, providers, hospitals, pharmaceutical firms, and patients would all face a transition period. Temporary disruptions, administrative complications, and unforeseen consequences should be expected.

Phased implementation is intended to identify and address these issues before full national adoption.

Known Limitations

This paper is a conceptual policy proposal rather than a completed healthcare financing model.

The proposals outlined here have not yet undergone comprehensive economic scoring, independent actuarial analysis, or full implementation modeling.

Several assumptions remain untested, including:

- Administrative cost savings.
- Behavioral responses.
- Provider participation.
- Long-term financing mechanisms.
- Healthcare utilization changes.
- Workforce impacts.
- Fiscal sustainability.

This paper should therefore be understood as a framework rather than a finished implementation plan.

Its purpose is to identify a potential path forward and invite criticism, analysis, and refinement.

Questions for Healthcare Economists and Policy Experts

This proposal is intended to invite scrutiny and informed criticism.

Particularly valuable feedback would include:

1. What assumptions appear weakest?
2. What unintended consequences are most likely?
3. How should comprehensive coverage be defined?
4. What provider-payment challenges are being overlooked?
5. What financing assumptions require further study?

6. How should supplemental private insurance interact with universal coverage?
7. What implementation risks appear most significant?
8. What evidence should policymakers require before considering adoption?
9. What mechanisms should prevent healthcare expenditures from growing faster than economic growth over the long term?
10. Can comprehensive coverage ultimately reduce total healthcare spending by shifting treatment toward preventative and early intervention services?
11. If Universal Health Insurance is not the preferred solution, what alternative framework would better achieve universal healthcare access without exposing individuals to financial ruin?

Conclusion

Healthcare affects every citizen.

The question is not whether healthcare costs exist.

The question is how those costs are distributed, financed, and managed.

This proposal begins with a simple principle:

Healthcare should be universal; no person should face financial ruin because of medical care.

Rather than viewing healthcare solely as an insurance problem, this framework treats healthcare access as a national economic, social, and public-health challenge.

It does not claim to provide every answer.

Instead, it seeks rigorous criticism, independent analysis, and constructive debate.

The purpose of this paper is not to end the healthcare discussion.

The purpose is to begin it.

Sources

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